

EVIDENCE FILE

CAN YOU DEFEND THE FINANCIAL RESULT?

Three source boundaries for a hospital AI renewal or expansion decision.

Use each source for the decision it can support

The evidence does not prove one composite hospital ROI claim. It separates payer spending, modeled capacity, and governance readiness.

SOURCE 1

Blue Cross Blue Shield Association coding-cost analysis

FIRST-PARTY PAYER ANALYSIS | SEPTEMBER 24, 2026

WHAT THE SOURCE SUPPORTS

- BCBSA estimated that higher-complexity hospital coding added \$942 million to spending by BCBS companies from 2023 through 2025.
- About 70 percent, or more than \$650 million, was tied to secondary diagnoses; BCBSA also reported that more than 60 percent of hospital systems use AI coding tools.

BOUNDARY

This is a payer finding about claims spending. The public release does not establish that AI caused the full amount, that every added diagnosis was improper, or that the result represents hospital net cash.

[OPEN PRIMARY SOURCE](#)

SOURCE 2

Distribution-aware operating-room scheduling study

PEER-REVIEWED STUDY | HEALTH CARE MANAGEMENT SCIENCE | SEPTEMBER 24, 2026

WHAT THE SOURCE SUPPORTS

- The study used historical data from three surgical platforms at one university health system and evaluated model-produced schedules offline.
- Average waiting times were 23 percent, 33 percent, and 66 percent lower than historical schedules across the three settings; utilization tradeoffs were also reported.

BOUNDARY

These are counterfactual scheduling results, not observed implementation outcomes or realized cash. The study notes that downstream constraints were not fully observable in the retrospective data.

[OPEN PRIMARY SOURCE](#)

Keep the financial claim connected to its control record

Governance does not prove the return. It preserves the inventory, version, risk review, and response history needed to defend the decision.

SOURCE 3

AAAHC AI governance materials for ambulatory organizations

FIRST-PARTY ACCREDITATION UPDATE | SEPTEMBER 25, 2026

WHAT THE SOURCE SUPPORTS

- The guide includes an AI risk assessment, AI system inventory, vendor evaluation tool, policy template, lifecycle checklist, and survey-readiness materials.
- AAAHC states that its v45 standards become effective on December 15, 2026.

BOUNDARY

The materials address AAAHC ambulatory accreditation and certification programs. Do not present them as a universal inpatient-hospital rule or substitute them for local legal, compliance, clinical, or financial review.

[OPEN PRIMARY SOURCE](#)

DECISION SYNTHESIS

Use the BCBSA analysis to define the coding and defensibility question. Use the operating-room study to distinguish modeled capacity from observed throughput and cash. Use the AAAHC materials to keep the approved use connected to its system inventory, version, risk review, and lifecycle record. The four-gate workbook is ALHQ editorial synthesis, not a research finding or legal rule.

APPROVAL PACKET CHECK

- One benefit claim and denominator are named.
- Baseline, comparison period, case mix, demand, staffing, policy, contract, and technology changes are documented.
- Value capture is shown as collected contribution or removed expense, net of incremental cash cost.
- Coding support, human review, claim disposition, payer response, exceptions, and version changes are traceable.
- Unknowns, escalation triggers, accountable owner, and recheck date are explicit.